

IMAGES IN CLINICAL MEDICINE

Pityriasis Rosea



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A PREVIOUSLY HEALTHY 42-YEAR-OLD MAN PRESENTED TO THE DERMATOLOGY clinic with a 20-day history of an expanding, asymptomatic rash on his trunk. The initial lesion had been a red spot on the patient's left side. Ten days after that spot had first appeared, smaller lesions had started developing elsewhere. The patient reported no viral prodrome. Physical examination was notable for multiple 1-to-4-cm ovoid, erythematous, papulosquamous plaques on the chest, anterior neck, and proximal upper arms (Panel A). The initial lesion — an 8.5-by-5.5-cm patch with central clearing and peripheral scaling — was seen on the midaxillary line on the left side (Panel B and inset). On the basis of this finding of a “herald” patch, a diagnosis of pityriasis rosea was made. Pityriasis rosea is a self-limiting disorder that typically affects children, adolescents, and young adults. Although the lesions often align along the cleavage lines of the skin in a “Christmas tree” pattern, they do not always manifest that way. The cause of pityriasis rosea remains unclear. It is not thought to be associated with bacterial or fungal organisms, although it may be related to viral reactivation of human herpes virus 6 or 7. Reassurance regarding the benign nature of the rash was provided. The rash resolved 1 month after onset.

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